

Cardiology Specialist Waitlist

Spec Driven Development Hackathon

Team 7



Our team

Named explicitly, not implied — this is who did what.



Fernanda Hernández

UX



Journey maps, decisions log, working HTML prototype



Julio Flores

BA

Product Spec authoring, user stories, acceptance criteria



Uriel Angeles

QA

Business rules review, edge cases, spec-vs-build alignment



Juan Carlos Abarca

BE / FE Dev

Data model, implementation feasibility, build support

The problem

A public hospital with eight specialties still runs scheduling by phone. Cardiology and oncology waitlists regularly exceed six weeks.

Our initial interpretation (before we found the full brief) assumed a single private clinic, one specialist. The real scope is one pilot specialty — cardiology — inside a much larger hospital system, with a meaningful share of patients who aren't digitally reachable at all.

What we know:

8

medical specialties hospital-wide

6+ wks

typical cardiology / oncology wait

40%

call-volume reduction goal

How we applied the pillars

Not the theory — what actually happened.

G

Governance

We overruled an AI suggestion to let urgent cases jump the waitlist — real clinics absorb emergencies as same-day delays, not queue reordering, so we locked in strict FIFO instead.

A

Alignment

UX built the journey maps and working prototype; BA/QA folded every open decision into the Product Spec — each round of feedback updated both before the next build pass.

Q

Quality

We caught our own prototype contradicting our own business rule — a decline was auto-cascading to the next patient when the spec required a separate staff action. Fixed before it reached the team.

L

Living Agility

A missed Confluence brief, found mid-hackathon, reframed our entire scope — from a single private clinic to one pilot specialty inside an 8-specialty public hospital — and the spec was rewritten to match, not the other way around.

A

Adaptability

We kept the prototype one HTML page with internal tabs instead of multiple pages, specifically because our dev needed to build fast — calibrated to the week's real constraint, not gold-plated.

Demo

The product in action — one shared record, two ways in: the web, or a phone call.

Dr. Elena Ruiz

Cardiology waitlist

Reset demo

Patient view

Staff view

Add a patient on their behalf:

Jorge Ramírez — calling in

Add to waitlist

Standing in for a patient lookup in the hospital's existing records system — that lookup already exists elsewhere and isn't built here.

Waiting on Carlos Mendoza's response — requires a call.

They accepted

They declined

Couldn't reach them — pass to next

#	Patient	Contact preference	Status
1	Carlos Mendoza	Telephone	Notified Requires a call
2	Ana Torres	Not recorded	Waiting

Patient-facing position isn't shown in this iteration (deferred to a future spec) — staff see full position order here; patients just see their own status.

What didn't go as expected

1

A brief we hadn't seen

On day 3 we discovered a Confluence doc with the full, real problem statement — our scope had been an assumption, not a fact, for two days.

2

A rule we'd built wrong

Our own prototype auto-advanced the waitlist on a decline — directly contradicting a business rule we ourselves had written. Caught by re-reading the spec against the build.

3

A bug only the script found

Writing the literal demo script — not just clicking around — surfaced a queue-ordering bug no one had noticed in casual testing.

The spec as a living record

What changed between day 1 and today — and why that's the point.

Day 1

- Single private clinic, one specialist — assumed
- Patients see a live waitlist position
- Patients can be removed from the list
- Digital-only — no telephone path considered

Today

- One pilot specialty in an 8-specialty hospital — confirmed
- Position display deferred — explicitly out of this iteration
- Removal deferred — explicitly out of this iteration
- Telephone path is a primary path, not an edge case

Thank you!

